

Clinical Guideline for: **Diabetes in Labour and the Postnatal Period** Eastern services

Summary

This guideline outlines the management of women with pre-existing diabetes in labour and the puerperium.

Key Points

- All diabetes is a high risk situation
- Increased risk of fetal distress and stillbirth
- Aim for glucose (capillary, blood or continuous glucose monitor) between 4-8 mmol/l during labour
- Insulin requirements drop to pre-pregnancy levels after delivery
- If on Insulin for >6 hours sodium levels will need to be monitored
- Gestational diabetes does not usually require insulin in labour and treatment can be stopped after delivery.

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1.0 INTRODUCTION

- 1.1 Good diabetic control in labour reduces the risk of fetal distress and neonatal hypoglycaemia (NICE 2008). This guideline outlines best practice to achieve this.

2.0 MOTHERS WITH TYPE 1 OR TYPE 2 DIABETES IN SPONTANEOUS LABOUR

2.1 On admission

Full antenatal admission assessment including CTG (NICE 2004) and blood glucose level (BGL).

On-call Registrar or consultant to review diabetic status and fetal condition.

- Review recent insulin requirement and glucose control. The aim is to keep blood glucose between **4-8mmol/l in labour** (The Joint British Diabetes Societies, 2023)
- Prescribed long acting and short acting insulin.
- Take blood for U&E, glucose, FBC, group and save
- A variable rate insulin infusion (VRII) will be required if vomiting, unwell or poorly controlled (glucose >14 mmol/l on admission or if blood glucose levels consistently remain above 10.0 mmol/L for >2hr despite taking additional correction dose of insulin taken by women). The blood glucose should be checked 2 hours after correction to prevent any delay in starting VRIII - see [Appendix A](#)
- If already on long-acting insulin, Continue long-acting insulin along with variable rate insulin infusion. Short acting insulin (and metformin if taken) should be hold off if VRIII is started. Do not use glucose from continuous glucose monitor for adjusting IV insulin dose. If on insulin pump, this will need to be stopped once on VRII.
- Review notes regarding discussion/decision of method of delivery.
- Inform diabetes specialist team at the most suitable time.

2.2 When in established labour

- Site IV cannula
- Take U&Es, LFTs, FBC, Group & Hold and baseline blood glucose if not taken before.
- Check capillary or continuous glucose monitor hourly until after delivery.
- A variable rate insulin infusion (VRII) will be required if vomiting, unwell or poorly controlled (glucose >14 mmol/l on admission or if blood glucose levels consistently remain above 10.0 mmol/L for >2hr despite taking additional correction dose of insulin taken by women). The blood glucose should be checked 2 hours after correction to prevent any delay in starting VRIII - see [Appendix A](#).
- If already on long-acting insulin, Continue long-acting insulin along with variable rate insulin infusion. Short acting insulin (and metformin if taken) should be hold off if VRIII is started. Do not use glucose from continuous glucose monitor for adjusting
- IV insulin dose. If on insulin pump, this will need to be stopped once on VRII.
- All women on insulin for >6 hours will require Sodium levels to be checked. Send a gold topped bottle and follow [Appendix B](#). Follow eating and drinking flowchart [Appendix C](#) for high risk labourer.
- Commence Omeprazole 20mg, orally 12-hourly in labour.

The aim of the regime is to achieve good control without massive swings in blood glucose.

Remember that high maternal blood glucose is important to address for the baby to lower the risk of fetal distress and neonatal hypoglycaemia (NICE 2008).

Advice can be obtained from the on-call diabetes specialist nurse from 9am to 8pm seven days a week but 1pm -8pm is for emergencies only. Ring the operator and ask for the diabetes specialist nurse on call.

3.0 INDUCTION OF LABOUR FOR WOMEN WITH PRE-EXISTING DIABETES (TYPE 1 OR TYPE 2)

3.1 The woman is admitted via FMAU on the day of induction. She should have her normal insulin and/or metformin and meals until established in labour. Check BGL before meals, before bed and other times if mother feels unwell (insulin requirements may go down in labour).

1. Propess 10mg vaginally to be given according to induction protocol.
2. 6 hourly CTGs to monitor baby
3. If not in labour after 24 hours, transfer to LW, reassess cervix and perform ARM
4. Recommend LSCS if still not possible to do ARM
5. When in labour give Omeprazole 20mg orally 12 hourly
6. A variable rate insulin infusion (VRII) will be required if vomiting, unwell or poorly controlled (glucose >14 mmol/l on admission or if blood glucose levels consistently remain above 10.0 mmol/L for >2hr despite taking additional correction dose of insulin taken by women. The blood glucose should be checked 2 hours after correction to prevent any delay in starting VRIII - see [Appendix A](#).
7. If already on long-acting insulin, Continue long-acting insulin along with variable rate insulin infusion. Short acting insulin (and metformin if taken) should be hold off if VRIII is started. Do not use glucose from continuous glucose monitor for adjusting IV insulin dose. If on insulin pump, this will need to be stopped once on VRII
8. If on insulin for over 6 hours take Sodium level by sending gold topped bottle and follow [Appendix B](#) flowchart for hyponatremia. Follow eating and drinking flowchart [Appendix C](#) for high risk labourer.
9. If vaginal delivery, may resume normal eating and drinking with pre-pregnancy doses of insulin, but these may have to be adjusted downwards if blood glucoses are low. If in doubt telephone the Diabetes team.

4.0 ELECTIVE CAESAREAN SECTION FOR MOTHERS WITH PRE-EXISTING DIABETES (TYPE 1 OR TYPE 2)

1. Book into first slot of the day
2. No need to admit the night before unless specific instructions in the notes
3. Advise normal evening short acting insulin and meal the night before, with bedtime snack and normal long acting insulin. Take metformin as normal if also on this. If taking twice a day long acting insulin, advice to take normal long acting insulin in the morning on the day of induction or caesarean section.
4. No change if on insulin pump, patient will self-manage and has already been advised by the diabetes team on suitable pump changes.
5. Omeprazole 20mg the night before (given at the pre-op visit).
6. Fast from midnight (obviously would need to treat hypoglycaemia in the night, but this could delay LSCS).
7. Admit to delivery suite for 8.00am.
8. Site IV cannula for fluids, take blood for U&E, and Group and Save.
9. Check glucose hourly using blood or continuous glucose monitor.
10. A variable rate insulin infusion (VRII) will be required if vomiting, unwell or poorly controlled (glucose >14 mmol/l on admission or if blood glucose levels consistently remain above 10.0 mmol/L for >2hr despite taking additional correction dose of insulin taken by women. The blood glucose should be checked 2 hours after correction to prevent any delay in starting VRIII - see [Appendix A](#).
11. If needed then a variable rate insulin infusion can be commenced (starting regime should be documented at last ANC visit and in Obstetric Management section of Epic. Aim for blood glucose in the range 4-8mmol/l.
12. If already on long-acting insulin, Continue long-acting insulin along with variable rate insulin infusion. Short acting insulin (and metformin if taken) should be hold off if VRIII is started Do not use glucose from continuous glucose monitor for adjusting IV insulin dose. Continue variable rate insulin infusion until eating and drinking normally.
13. Change the insulin dose to pre-pregnancy dose post-delivery but be prepared to adjust downwards if blood glucoses low.

5.0 EMERGENCY CAESAREAN SECTION FOR MOTHERS WITH PREEXISTING DIABETES (TYPE 1 OR TYPE 2)

- Site IV cannula for fluids if not already present, take blood for U&E, and Grp+S.
- Check blood glucose hourly.
- Give IV Omeprazole 50mg over 2 mins.
- If needed, then a variable rate insulin infusion can be commenced (starting regime should be documented at last ANC visit and in Obstetric Management section of Epic. Aim for blood glucose in the range 4-8mmol/l.
- If already on long-acting insulin, Continue long-acting insulin along with variable rate insulin infusion. Short acting insulin (and metformin if taken) should be hold off if VRIII is started Do not use glucose from continuous glucose monitor for adjusting IV insulin dose.

- Continue variable rate insulin infusion until eating and drinking normally.
- Prescribe pre-pregnancy dose of insulin post-delivery but be prepared to adjust downwards if blood glucose low.

6.0 INSULIN PUMPS IN LABOUR

- 6.1 Increasing numbers of women with diabetes use an insulin pump rather than standard injections. These pumps are complex and must only be used if the woman or her partner are fully capable of using them during labour.
- 6.2 We encourage all women to continue insulin pump in labour as it has been shown to be safe and effective way to control intrapartum glycaemia including during caesarean section.
- 6.3 If a woman becomes unwell, unconscious, unable to use the pump or requests that the pump is stopped then the insulin pump should be discontinued and a variable rate insulin infusion set up (See [Appendix A](#)).
- 6.4 In the case of a Caesarean section, the anaesthetist should discuss the continued use of the pump with the patient and a decision made whether it should be discontinued or not. It is sensible to advise pump placement more laterally on the abdomen at term to avoid the skin area cleaned for a Caesarean.

7.0 POST NATAL CARE

- 7.1 After giving birth the mothers' insulin requirements drop to the pre-pregnancy dose but control may be unstable owing to increased insulin sensitivity.
 - **Aim to keep BG between 6-10 mmol/l to avoid hypoglycaemia when looking after the baby.**
 - Inform the diabetes specialist nurse of delivery
 - Baby to be checked for hypoglycaemia (see protocol)
 - Breastfeeding should be encouraged. The baby should feed as soon as possible after birth, preferably within 30 minutes then frequently every 2-3 hours to maintain pre-feed capillary glucose levels at 2.0mmol/L or above. Supplementation may be required if baby becomes hypoglycaemic (see neonatal guideline). The mothers should have had the option to try to express and store breast milk prior to delivery in case they are separated from their newborn baby.
 - Advise the mother that once breastfeeding is established insulin requirements may drop below pre-pregnancy dose.
 - **Insulin doses may need to be reduced by 30% if breast feeding.**
 - Metformin and Glibenclamide are thought to be safe for breast feeding mothers, also Ramipril and Trandolapril (ACE inhibitors) for hypertension. (NICE 2008 and 2010).

7.2 Follow Up

- Offer follow up with diabetes team in the diabetes centre in 2-3 months.
- Give pre-conceptual care leaflet
- Discuss contraceptive requirements

8.0 MANAGEMENT OF GESTATIONAL DIABETES IN SPONTANEOUS LABOUR

8.1 On admission

Full antenatal admission assessment including CTG (NICE 2004) and blood glucose level (BGL).

On call Registrar or consultant to review diabetic status and fetal condition.

- Take blood for U&E, glucose, FBC, group and save.
- A variable rate insulin infusion (VRII) will be required if vomiting, unwell or poorly controlled (glucose >14 mmol/l on admission or if blood glucose levels consistently remain above 10.0 mmol/L for >2hr despite taking additional correction dose of insulin taken by women. The blood glucose should be checked 2 hours after correction to prevent any delay in starting VRIII - see [Appendix A](#).
- If already on long-acting insulin, Continue long-acting insulin along with variable rate insulin infusion. Short acting insulin (and metformin if taken) should be hold off if VRIII is started. Do not use glucose from continuous glucose monitor for adjusting IV insulin dose.
- Commence Omeprazole 20mg orally 12 hourly.
- Inform diabetic specialist team at the most suitable time.

8.2 Gestational diabetes controlled on diet

- Monitor 4 hourly blood glucose throughout labour and once post-delivery.
- Insulin is not normally required during labour.

8.3 Gestational diabetes controlled on diet and metformin

- Monitor 4 hourly blood glucose throughout labour and once post-delivery.
- Insulin is not normally required during labour.
- Stop metformin after delivery.

8.4 Gestational diabetes requiring insulin during pregnancy

(Usually requiring insulin once or twice daily)

- Take normal insulin and meals in early labour.
- Insulin infusion not normally required in labour as not eating.
- Hourly blood glucoses in labour and check once after delivery.
- A variable rate insulin infusion (VRII) will be required if vomiting, unwell or poorly controlled (glucose >14 mmol/l on admission or if blood glucose levels consistently remain above 10.0 mmol/L for >2hr despite taking additional correction dose of insulin taken by women. The blood glucose should be checked 2 hours after correction to prevent any delay in starting VRIII - see [Appendix A](#).
- If already on long-acting insulin, Continue long-acting insulin along with variable rate insulin infusion. Short acting insulin (and metformin if taken) should be hold off if VRIII is started. Do not use glucose from continuous glucose monitor for adjusting IV insulin dose ([Appendix A](#)).
- Normally do not require insulin after delivery.

9.0 INDUCTION OF LABOUR FOR WOMEN WITH GESTATIONAL DIABETES

The woman is admitted to FMAU on the day of induction as booked and later transferred to the ward. She should have her normal insulin and/or metformin and meals until established in labour. Check BGL before meals, before bed and other times if mother feels unwell.

- Propess to be given according to induction protocol.
- 6 hourly CTGs to monitor baby
- If not in labour after 24 hours reassess cervix and perform ARM
- Recommend LSCS if still not possible to do ARM
- When in labour give Omeprazole 20mg orally 12 hourly
- A variable rate insulin infusion (VRII) will be required if vomiting, unwell or poorly controlled (glucose >14 mmol/l on admission or if blood glucose levels consistently remain above 10.0 mmol/L for >2hr despite taking additional correction dose of insulin taken by women. The blood glucose should be checked 2 hours after correction to prevent any delay in starting VRIII - see [Appendix A](#).
- If already on long-acting insulin, Continue long-acting insulin along with variable rate insulin infusion. Short acting insulin (and metformin if taken) should be hold off if VRIII is started. Do not use glucose from continuous glucose monitor for adjusting IV insulin dose ([Appendix A](#)).
- If vaginal delivery, may resume normal eating and drinking. Usually do not require insulin or metformin postnatally.

10.0 ELECTIVE CAESAREAN SECTION FOR MOTHERS WITH GESTATIONAL DIABETES

1. BOOK INTO FIRST SLOT OF THE DAY
2. Advise normal evening insulin/metformin and meals the night before.
3. Give routine oral Omeprazole 20mg the night before
4. Fast from midnight
5. Admit to delivery suite for 8.00 am
6. Site IV cannula for fluids, take blood for U&E, and Group and Save.
7. Check blood glucose
8. A variable rate insulin infusion (VRII) will be required if vomiting, unwell or poorly controlled (glucose >14 mmol/l on admission or if blood glucose levels consistently remain above 10.0 mmol/L for >2hr despite taking additional correction dose of insulin taken by women. The blood glucose should be checked 2 hours after correction to prevent any delay in starting VRIII - see [Appendix A](#).
9. If already on long-acting insulin, Continue long-acting insulin along with variable rate insulin infusion. Short acting insulin (and metformin if taken) should be hold off if VRIII is started. Do not use glucose from continuous glucose monitor for adjusting IV insulin dose.

11.0 EMERGENCY CAESAREAN SECTION FOR MOTHERS WITH GESTATIONAL DIABETES

1. Site IV cannula for fluids if not already present, take blood for U&E, and Group and Save.
2. Check blood glucose.
3. Give IV Omeprazole 50mg over 2 mins.
4. Variable rate insulin infusion (VRII) will be required if vomiting, unwell or poorly controlled (glucose >14 mmol/l on admission or if blood glucose levels consistently remain above 10.0 mmol/L for >2hr despite taking additional correction dose of insulin taken by women. The blood glucose should be checked 2 hours after correction to prevent any delay in starting VRIII - see [Appendix A](#).
5. If already on long-acting insulin, continue long-acting insulin along with variable rate insulin infusion. Short acting insulin (and metformin if taken) should be hold off if VRIII is started. Do not use glucose from continuous glucose monitor for adjusting IV insulin dose.
6. Check BGL 4 hours after delivery.

12.0 POST NATAL CARE FOR WOMEN WITH GDM

- Monitoring of blood glucoses can stop.
- Metformin and insulin can be stopped
- Inform the diabetes specialist nurse of delivery
- Check baby for hypoglycaemia as per protocol

13.0 FOLLOW UP FOR WOMEN WITH GDM

- Recommend HbA1C 13 weeks postnatally by GP at routine postnatal visit to check for type 2 diabetes and HbA1C annually thereafter.
- Give pre-conceptual care leaflet
- Discuss contraceptive requirements

14.0 MONITORING COMPLIANCE WITH THIS GUIDELINE

14.1 Any concern or non-compliance with this guideline that is identified through the investigation of clinical incidents, claims or complaints will be reviewed as per the Trust Policies regarding Incidents, Claims and Complaints, and may result in an audit and/or amendment to the guideline.

14.2 Relevant Policies:

- Incident reporting policy and procedure
- Claims management policy and procedure
- Policy and Procedure for the Management of Complaints, Concerns, Comments and Compliments

15.0 ASSOCIATED CLINICAL GUIDELINES OR POLICIES

15.1 Clinical Guideline for the Management of Diabetes in Pregnancy – Antenatal Care

16.0 PUBLICATION DETAILS

Author of Clinical Guideline	Jenny Blackman O&G Consultant
Directorate/Department responsible for Guideline	Women's and Children's Health / CWH / Maternity
Contact details	X6590
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APPENDIX A - INTRAVENOUS INSULIN CHART AND INFUSION GUIDANCE (MATERNITY ONLY)

INTRAVENOUS INSULIN CHART (MATERNITY PATIENTS ONLY)

(continued on reverse)

This form is only to be used in conjunction with the labour ward guidelines

INSULIN INFUSION GUIDELINES

50 units of Human Actrapid in 49.5mls 0.9% sodium chloride.

Aim for target BGL 4-8 mmol/L until after delivery

Aim for target BGL range 4 – 10 mmol/L postnatally

This must be run concurrently with IV glucose

Standard regimen: 10% dextrose 500 mls, 10 mmol/KCl at 50 mls/hr (record batch no. on fluid chart)

Alternative regimens: Discuss with Obstetric and Diabetic team

Achieving BGL target range

Start at 2 units Actrapid per hour, and continue until target range achieved.

If BGL stays the same or rises on 2 consecutive readings, increase Actrapid infusion rate by 2 units /hr.

Maintaining target BGL range. (See labour ward guidelines)

If BGL rises above target range on 2 consecutive readings, increase Actrapid infusion rate by 1 unit /hr.

If BGL falls below target range, decrease Actrapid infusion rate by 1 or 2 units /hr.

Check Blood Glucose levels every hour.

Affix patient ID sticker

Date :

Signature :

Print name:

Bleep No.

Date & Time	Capillary blood glucose	Rate of insulin infusion Units per hour	Indicate time and sign when rate changes	Hourly check of IV site & rate		Comments
				Vol left in syringe	Signature & initials	

[illegible]

APPENDIX B - SYMPTOMS AND TREATMENT FOR HYPOGLYCAEMIA

HYPOGLYCAEMIA

Signs & Symptoms

Difficulty in concentrating
Sweating
Dizziness
Tremor
Tingling (Hands, feet or tongue)
Palpitations
Headache
Hunger
Blurred vision
Low blood glucose reading

Most diabetics experience 3 or 4 warning symptoms and can usually recognise these.

If untreated and blood glucose levels continue to fall

Irrational or aggressive behaviour
Amnesia
Progressive confusion Drowsiness

If still untreated

Convulsions
Loss of consciousness
Possible death

If untreated and blood glucose levels continue to fall

Irrational or aggressive behaviour
Amnesia
Progressive confusion Drowsiness

If still untreated

Convulsions
Loss of consciousness

Possible death

Treatment of Hypoglycaemia (See below)

Always treat hypos immediately
Ideally perform blood glucose test to ascertain level

If the Patient is Unconscious or Not Able to Swallow Safely:

This is an emergency situation requiring urgent intervention. GlucoGEL should be avoided

Glucagon injection 1mg intramuscularly should be delivered. This is usually effective within 10 minutes

If Glucagon injection is ineffective or contraindicated

20 - 50% dextrose can be considered by bolus injection into a large vein or intravenous infusion, administered by a Doctor only, once prescribed.

Caution regarding extravasation

Once consciousness has returned the patient should be given a glucose drink and then additional slower-acting carbohydrate to maintain blood glucose level

Recovery may be delayed if:

Prolonged/severe hypoglycaemia

Alternative cause i.e. CVE, drug overdose,

Head injury (caused by the hypo)

Patient is post-ictal (convulsion caused by hypo)

If the Patient Is Conscious, Co-Operative and Able to Swallow Safely

Best treatment is rapid acting carbohydrate (10-20g):

Lucozade (100-130mls / approx. half cup)

Fruit juice (150-200mls / approx. 1 cup or small carton)

Lemonade or cola (150-200mls)

Glucose tablets – good but take slightly longer to work and 5 – 6 tablets are required to correct hypoglycaemia.

With these hypo treatments, if symptoms do not improve within 5 minutes, repeat treatment and seek medical advice

If hypoglycaemia occurs within 1 hour of the next meal then rapid-acting carbohydrate is usually all that is necessary. If more than 1 hour until the next meal then take by slower acting carbohydrates.

Bread/toast

Milk and biscuits

Cereal bar

Chocolate, biscuits, milk and other foods are not recommended as first line treatments for hypoglycaemia because the fat content in these foods delays the digestion of glucose.

If the patient is conscious and able to swallow safely but uncooperative consider the use of Glucogel and seek advice from supervising clinician.

TO USE GLUCOGEL

Insert Gel into patient's mouth and massage gently around cheeks. The Gel is absorbed into the buccal mucosa.

APPENDIX C- Eating and Drinking

Eating and Drinking in labour and before an ELCS



BACKGROUND

Low risk women should be encouraged to eat a light diet during labour. A systematic review of food intake during labour found women who eat during labour have significantly shorter duration of labour and have improved satisfaction.

In high risk women this must be balanced with the risk of vomiting and aspiration. These should be managed on an individual basis based on the specific risk factors and discussed with the obstetric team (Obstetricians and/or Anaesthetists).

Fluids in both low and high risk groups should ideally be isotonic and non carbonated such as Lucozade sport, Oasis or Powerade. These contain glucose to provide energy and have additional electrolytes that reduce the occurrence of hyponatraemia in labour.

Low Risk Labour in all settings

(The presence of an epidural without additional risk factors is considered low risk)

Suggest light Diet and Recommended fluids

High risk labour

- e.g.
- Breech presentation
 - Multiple pregnancy
 - Severe pre-eclampsia
 - Previous C-section or uterine scar
 - Hx of PPH (>1L) due to atony/coagulopathy or retained placenta
 - Booking BMI >40
 - Pathological CTG
 - Fetal blood sampling
 - Thick meconium stained liquor
 - Significant antepartum haemorrhage

(This list is not exhaustive and if uncertain discuss with Obstetric team on the Senior Ward Round for individual management)

- Recommended fluids
- Low residue foods
- Consider omeprazole 20mg bd

Elective C-Section

- Eat until 02:00 on the morning of surgery
- Drink until arrival at the Hospital
- Omeprazole 20mg on the morning of surgery
- Sips of water until called to theatre.

Sip til you send!
 (~50mls clear fluid/hr until called to theatre)

Postpartum

Normal Vaginal Delivery, uncomplicated instrumental delivery or C-Section

- No restriction on drinking and light diet.
- Avoid eating until 1 hour post-operatively

Complicated vaginal delivery or C-Section

• If high risk of return to theatre then diet to be confirmed with the obstetric team.
 Sips of water if in doubt.

On return to ward: No restriction on eating or drinking.

Light diet

- Sandwich
- jacket potato
- biscuit
- cereals
- toast
- jelly
- ice lollies
- jelly sweets
- yoghurt
- ice cream



Low residue foods

- Jelly
- ice cream
- yoghurt
- jelly sweets



Recommended fluids

Drink to thirst!

- Ideally isotonic sport non fizzy drinks (i.e. Lucozade sport/ Powerade/ Oasis)
- Carbohydrate preoperative drinks
- Clear fluid (i.e. squash)
- Small coffee/tea with small amount of milk
- water